Pembrolizumab + pemetrexed + cisplatin

NCCP 00569 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Appointments and time in the unit

Pembrolizumab first over 30 minutes, then pemetrexed over 10 minutes. Cisplatin starts 30 minutes after pemetrexed and runs over 2 hours; pre/post hydration adds time during induction. Clinic time includes assessment, preparation and waiting; ask your unit about the full visit.

STEP 2

Treatment phase

Induction: all three medicines on Day 1 every 21 days for up to four cycles. Maintenance: cisplatin stops; pembrolizumab and pemetrexed may continue every 21 days while appropriate. The team agrees duration and review points.

STEP 3

Medicines and practical help

Bring the hydration, anti-sickness, folate, B12 and steroid plan. Ask who organises B12 and blood tests; check fluid advice for your own health. Keep the immunotherapy alert card and urgent numbers with you.

STEP 4

After treatment

Response imaging follows the consultant plan; no fixed universal scan interval is specified in these NCCP regimens. Immune toxicity can arise after the final pembrolizumab dose. Continue symptom vigilance, immunotherapy alert card and the team’s follow-up plan. Continue folic acid during pemetrexed and for 21 days after its last dose; B12 every three cycles while pemetrexed remains active.

Sources: NCCP 00569 v5 · official regimen. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.